CLINICAL INTELLIGENCE TREND REPORT

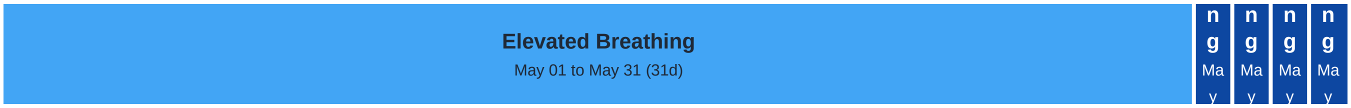
RED: Provider Review Recommended

Patient ID: Wheeler **Period:** May 1 to June 1, 2026 **Report Date:** June 2, 2026 **Coverage:** 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

HR Breathing



Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: RED

Vital	Avg	Min	Max
Heart Rate	59 bpm	51 bpm	71 bpm
Breathing	22 brpm	12 brpm	51 brpm

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 31	2 AM to 7 AM	5h	1	**Elevated Breathing**	26	14	**51**	Check SpO2 and lung sounds; assess for fluid overload or early infection

77 episodic events Detected, spanning 11 days total. Conditions: Elevated Breathing, High Breathing. **Priority grade: High**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
77	278h	Elevated Breathing	2	100%

Major Findings: Sustained elevated breathing (avg 26, peak 51)

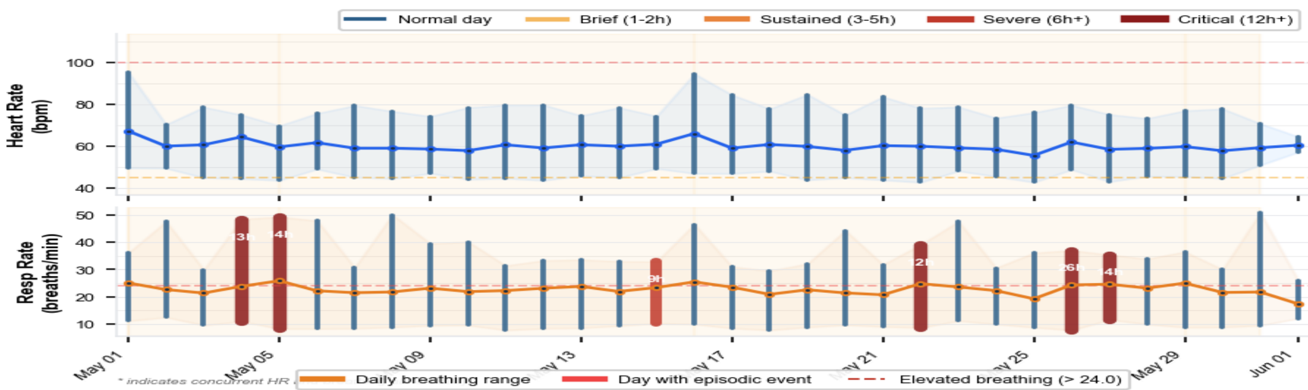
Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 01	7 AM to 11 AM	4h	1	**High Breathing**	31	17	**34**	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	9 PM to 10 PM	1h	1	**High Breathing**	32	12	**49**	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 16	8 AM to 9 AM	4h	2	**High Breathing**	31	17	**36**	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 29	6 AM to 7 AM	1h	1	**High Breathing**	32	27	**36**	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 01	3 AM to 7 AM	268h	72	**Elevated Breathing**	26	8	**51**	Check SpO2 and lung sounds; assess for fluid overload or early infection

Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

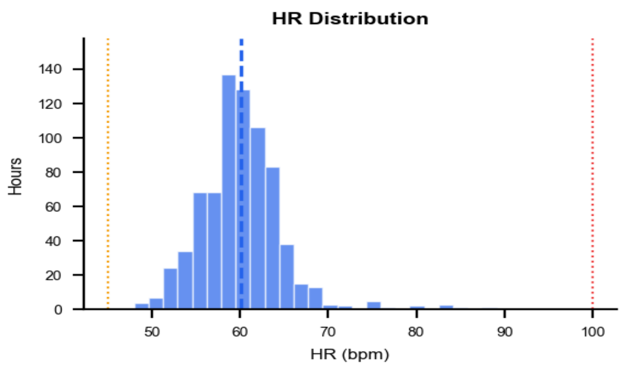
- Elevated Breathing (avg 26 brpm, peak 51 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 34 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 5 unstable phases with 13 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

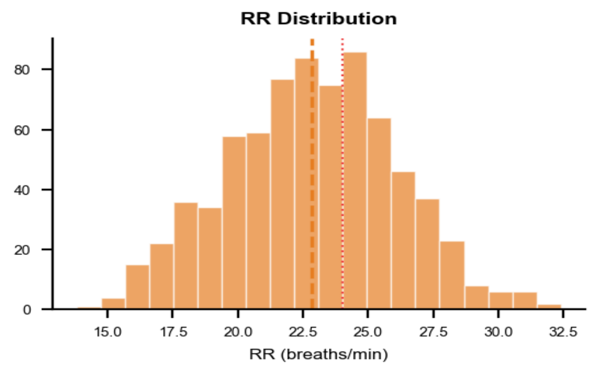
Clinical Pattern Observations:
1. **Sustained finding:** 14h continuous Elevated Breathing on May 05.
2. **Breathing variability:** RR spread 10 brpm (17 to 27).
3. **Clustered pattern:** Episodic events on 6 of 32 days (18%).



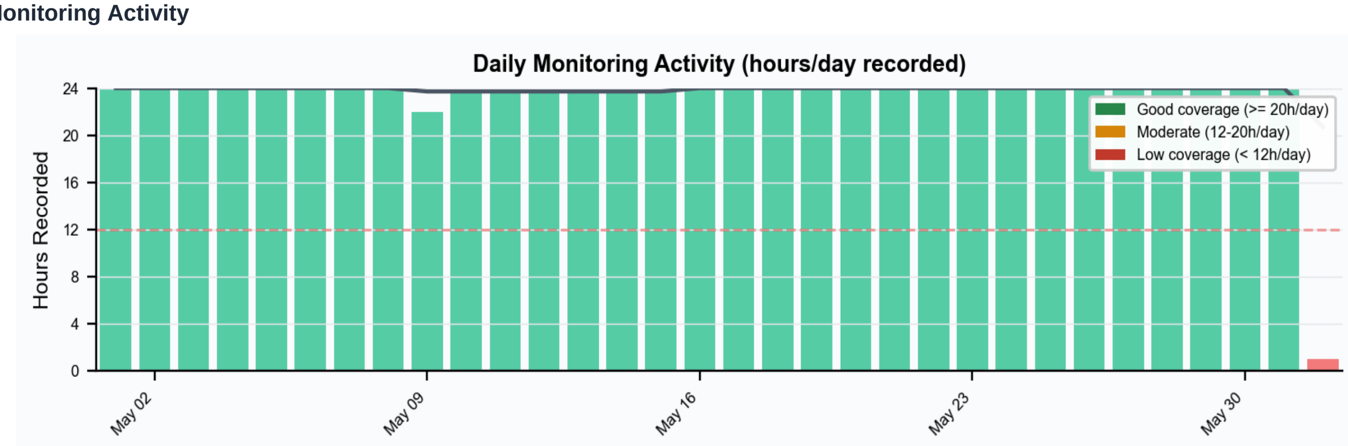
Red bars indicate days with detected episodic events.

Vital Sign Distribution





Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode *RR episode* *Recorded, no episode* *No data*

Clinical Alerting Thresholds

Very Low HR *< 40 bpm*	***Low HR*** *< 45 bpm*	***Elevated HR*** *> 95 bpm*	***High HR*** *> 100 bpm*
Very High HR *> 110 bpm*	***Elevated Breathing*** *> 24 brpm*	***High Breathing*** *> 30 brpm*	***Very High Breathing*** *> 40 brpm*

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.